



Kanangra Court Aged Care

Medication Management Policy and Procedure

Prescribing, supply, administration, review and reporting of medicines

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Document control

This document is controlled by the Director of Nursing. Printed copies are uncontrolled and must be checked against the document register before use. Proposed amendments are submitted to the document owner and considered by the Clinical Governance Committee at its next scheduled meeting.

Version history

Version	Date	Author	Summary of change
7.0	18 Apr 2024	P. Nadarajah	Introduced electronic medication charts across all houses.
8.0	6 Mar 2025	P. Nadarajah	Added the psychotropic self-assessment and quarterly reporting requirements.
9.0	20 Jul 2026	H. Okafor	Rewritten against the Strengthened Standards; added deprescribing at Part 5.
9.1	2 Sep 2026	H. Okafor	Clarified the nurse-initiated medicines list and the schedule 8 count frequency.

Distribution and accessibility

The current version is published on the provider intranet and is available in hard copy at each nurses' station. Alternative formats, including large print and interpreted summaries, are provided on request to the document owner.

Audience	Method	Frequency
All care and clinical workers	Intranet and induction pack	On publication
Registered and enrolled nurses	Clinical handover briefing	On publication
Consumer Advisory Body	Emailed summary	Quarterly
Contracted and agency staff	Agency portal	Before first shift

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Part 1 Purpose, principles and scope

This document governs every stage of a medicine's journey at Kanangra Court Aged Care: prescribing, supply, storage, administration, monitoring, review and disposal. It gives effect to Standard 5 of the Strengthened Aged Care Quality Standards and to the provider's obligations under State poisons legislation.

The guiding principle is that every medicine a person takes should have a current, documented reason to be taken. A medicine without an indication is a risk without a benefit. The provider is committed to reducing unnecessary medicines, and in particular to minimising the use of psychotropic medicines as a response to changed behaviour.

1.1 Scope

This document applies to prescription medicines, non-prescription medicines, complementary medicines, dietary supplements and medicines supplied by a person's family. It applies to registered and enrolled nurses, to care workers who have been assessed as competent to assist with medicines, and to visiting medical and pharmacy practitioners.

1.2 Who may administer a medicine

Worker	May administer	May not
Registered nurse	All routes and schedules within scope of practice	Medicines outside their competency assessment
Enrolled nurse (with notation)	Oral, topical, inhaled, eye and ear; subcutaneous where assessed	Intravenous medicines; Schedule 8 without RN check
Care worker (assessed)	Assist with pre-packed oral doses from a unit-dose pack	Injections, variable doses, crushed or altered doses, PRN psychotropics
Person receiving care	Self-administer under an agreed self-administration plan	Storage outside a lockable space where risk to others exists

1.3 Right to refuse

A person may refuse any medicine at any time. Refusal is recorded, the reason explored where the person is willing, and the prescriber notified where refusal is repeated or clinically significant. Medicines must never be concealed in food or drink except under a current covert administration authority approved under clause 4.4.

Part 2 Prescribing and the medication chart

A medicine may be administered only against a current, legible and complete order made by an authorised prescriber. Verbal orders are permitted only in the circumstances set out at 2.3 and must be confirmed in writing within 24 hours.

2.1 Requirements of a valid order

- The person's full name, date of birth and unique identifier.
- The medicine's active ingredient name, form, strength, dose, route and frequency.
- The indication for the medicine, recorded in every case.

- A start date and, for any medicine intended to be time-limited, a stop or review date.
- For a PRN medicine, the maximum dose in 24 hours and the specific circumstances of use.
- The prescriber's signature, printed name, provider number and the date.

2.2 Reviewing the chart

A registered nurse checks every new or amended chart against the previous chart and the person's medicine list before the first dose is given. Any discrepancy is resolved with the prescriber before administration; it is never resolved by assumption. The check is recorded with the nurse's name and the time.

2.3 Verbal and telephone orders

A verbal order may be accepted only where a delay would harm the person and the prescriber cannot attend. Two registered nurses must hear the order, one must read it back to the prescriber, and both must sign the chart. The prescriber must sign the chart within 24 hours; where they do not, the Director of Nursing is notified and the medicine ceases until an order is obtained.

2.4 Nurse-initiated medicines

A registered nurse may initiate a medicine appearing on the approved nurse-initiated list at Appendix F, for a maximum of 24 hours, where the clinical indication is met. The initiation is documented, the prescriber is notified at the next contact, and the medicine ceases at 24 hours unless charted.

Part 3 Storage, supply and disposal

Medicines are stored securely, at the temperature the manufacturer specifies, and in a manner that prevents access by any person other than an authorised worker.

3.1 General storage

Medication rooms and trolleys are locked whenever unattended, without exception. Keys are carried by the nurse in charge and never left in a lock. Refrigerated medicines are stored between 2 and 8 degrees Celsius in a dedicated medicines refrigerator with a continuous temperature log; the refrigerator is not used for food, specimens or vaccines belonging to workers.

3.2 Schedule 8 medicines

- Stored in a fixed, separately keyed safe within a locked room.
- Counted at every shift change by two workers, at least one a registered nurse, and the count signed in the register.
- Administered with a second-person check of the person, medicine, dose, route and time.
- Any discrepancy reported to the Director of Nursing immediately and, where theft or diversion is suspected, to police and the Commission within 24 hours.
- Register retained for the period State legislation requires, and never altered by overwriting; corrections are made by a dated single-line strike-through and initials.

3.3 Temperature excursions

Where a refrigerator log shows a temperature outside the 2 to 8 degree range, the affected medicines are quarantined and not administered until the pharmacist advises on stability. The excursion is recorded as an

incident. Refrigerator logs are reviewed weekly by the nurse in charge and monthly by the Director of Nursing.

3.4 Disposal

Ceased, expired and returned medicines are placed in the pharmacy return bin and collected under the Return of Unwanted Medicines arrangements. Schedule 8 medicines are destroyed only in the presence of two authorised workers with the destruction recorded in the register. Medicines are never disposed of in general waste or to sewer.

Part 4 Administration

Administration is the point at which most preventable harm occurs, and the checks in this Part are mandatory on every occasion, for every dose, regardless of familiarity with the person or the medicine.

4.1 The administration check

- Right person, confirmed by photograph and by asking the person to state their name where able.
- Right medicine, confirmed against the chart by active ingredient name.
- Right dose, including a second-person check for insulin, anticoagulants, opioids and any variable dose.
- Right route and right time, within one hour either side of the charted time.
- Right documentation, signed immediately after administration and never in advance.

4.2 Altering a dose form

A tablet may be crushed or a capsule opened only where the pharmacist has confirmed in writing that the medicine may be altered, and the authority is recorded on the chart. Modified-release and enteric-coated preparations must never be crushed. Where a person cannot swallow a solid dose form, the prescriber is asked to consider an alternative preparation.

4.3 Medication incidents

A medication incident includes a wrong medicine, wrong dose, wrong person, wrong route, wrong time beyond the permitted window, an omitted dose, and a dose given against an invalid order. Every medication incident is recorded, the person is assessed, the prescriber is notified where clinically indicated, and the incident is reviewed at the monthly medication safety meeting.

4.4 Covert administration

Covert administration is permitted only where the person lacks capacity for the decision, the substitute decision-maker has consented in writing, the prescriber and pharmacist have both approved the method, and the authority has been reviewed within the last three months. The authority must be reviewed again by 31 January 2027 for every person for whom one is currently in force.

Part 5 Review, deprescribing and psychotropic medicines

Every person's medicines are reviewed on admission, at every change in condition, after every hospital transfer, and at least every six months. The purpose of review is to confirm that each medicine still has an indication and that its benefits still outweigh its harms.

5.1 Residential Medication Management Review

A Residential Medication Management Review by an accredited pharmacist is arranged within 12 weeks of admission and at least every two years thereafter, and sooner following a fall, a significant change in condition or the addition of a high-risk medicine. Recommendations are discussed with the prescriber and the person, and the outcome of each recommendation is recorded, including where it is not accepted and why.

5.2 Psychotropic medicines

A psychotropic medicine must not be used as a first response to changed behaviour. Non-pharmacological strategies must be tried and documented first, unless there is an immediate risk of serious harm. Where a psychotropic is prescribed for changed behaviour it is a restrictive practice, and the requirements of the Restrictive Practices Policy apply in addition to this document.

Requirement	Timeframe	Responsible
Informed consent obtained before the first dose	Before administration	Prescriber
Behaviour support plan in place	Within 5 days of first administration	Clinical Nurse Consultant
Documented review of continuing need	At least every 3 months	Prescriber and Director of Nursing
Report psychotropic use to the National Quality Indicator Program	Quarterly	Quality Coordinator
Provider-wide psychotropic self-assessment	By 31 March 2027	Director of Nursing

5.3 Deprescribing

Where a medicine no longer has a clear indication, the prescriber is asked to consider deprescribing. Deprescribing is planned, staged and monitored: the person and their supporter are involved, one medicine is usually changed at a time, and the person is observed for withdrawal effects and for the return of the symptom the medicine was treating.

Part 5A Antimicrobial stewardship

Antimicrobial resistance is a risk to every person in the service, not only to the person prescribed the antimicrobial. The provider participates in antimicrobial stewardship to ensure antimicrobials are used only where indicated, at the right dose and for the shortest effective duration.

5A.1 Before an antimicrobial is commenced

- A clinical assessment is documented, including observations and the suspected source of infection.
- Specimens for culture are collected before the first dose where practicable.
- The indication, the intended duration and the review date are recorded on the chart.
- Asymptomatic bacteriuria is not treated; a positive urine culture without symptoms is not an indication.

5A.2 Review and duration

Every antimicrobial is reviewed within 48 to 72 hours of commencement against the culture result and the person's response, and the review is documented. A course without a documented stop date is escalated to the prescriber by the nurse in charge. Surveillance data is reported to the Medication Advisory Committee quarterly, and the annual stewardship report is due by 31 July 2027.

Part 6 Governance and accountability for medication management

The Board of Kanangra Court Aged Care holds ultimate accountability for medication management and discharges that accountability through the Medication Advisory Committee. The Board receives a standing report at each ordinary meeting covering performance against the indicators set out in Part 9, together with any matter the Director of Nursing considers requires the Board's attention before the next reporting cycle.

The Medication Advisory Committee meets monthly. Its quorum is four members and must include at least one registered nurse and one member independent of the operational management line. Minutes are retained for seven years and are made available to the Aged Care Quality and Safety Commission on request.

6.1 Allocation of responsibilities

Responsibility for the requirements of this document is allocated as set out below. Allocation of a responsibility does not relieve any worker of their own professional and legal obligations.

Role	Responsibility
Board	Monitors medication safety indicators and psychotropic use rates quarterly.
Director of Nursing	Owens this document; approves competency; reviews incident trends and refrigerator logs monthly.
Medication Advisory Committee	Reviews incidents, stewardship data and formulary matters; meets monthly.
Clinical Nurse Consultant	Ensures behaviour support plans accompany psychotropic use; supports deprescribing.
Registered nurses	Check charts before first dose; perform second-person checks; complete Schedule 8 counts.
Care workers (assessed)	Assist with pre-packed oral doses only; report refusals, omissions and errors immediately.
Consultant pharmacist	Conducts medication reviews; advises on crushing, stability and temperature excursions.

6.2 Delegation

The Director of Nursing may delegate an operational function under this document in writing, recording the scope and duration of the delegation in the delegations register. A delegation must not exceed twelve months without review, and the delegate must hold the qualifications and current registration the function requires. The Director of Nursing remains accountable for a delegated function.

6.3 Conflicts of interest

A worker who has a personal, financial or familial interest that could reasonably be perceived to influence a decision made under this document must declare that interest to the Director of Nursing before taking part in the decision. Declared interests are recorded in the conflicts register and reviewed by the Medication Advisory Committee each quarter.

Part 7 Workforce competence and training

No worker may perform a function governed by this document until they have completed the training set out in this Part and been assessed as competent. The Director of Nursing maintains a training matrix mapping every role to its mandatory modules, and the workforce management system prevents rostering a worker to a function for which their competency has lapsed.

Module	Audience	Frequency	Assessment
Safe administration of medicines	All workers who administer	Annually	Direct observation of 5 administrations
Schedule 8 handling and register	Registered and enrolled nurses	Annually	Register audit and observation
Psychotropic medicines and restrictive practices	All clinical workers	Annually	Scenario assessment
Antimicrobial stewardship	Registered nurses and prescribers	Every two years	Online assessment
Insulin and anticoagulant safety	Registered and enrolled nurses	Annually	Calculation test, 100% pass

7.1 Induction

New workers complete the medication management component of induction before their first unsupervised shift. Induction covers this document, the escalation pathway in Part 5, and the worker's obligation to report. A worker who has not completed induction may work only under the direct supervision of a competent worker.

7.2 Competency assessment and remediation

Competency is assessed by direct observation against the criteria at Appendix B, not by attendance alone. A worker assessed as not yet competent is placed on a supervised practice plan for a defined period, reassessed at the end of that period, and stood down from the relevant function if the second assessment is not satisfactory.

The Director of Nursing reports training completion rates to the Medication Advisory Committee monthly. Where completion for any mandatory module falls below 95 per cent, the Director of Nursing must provide the Medication Advisory Committee with a written recovery plan at the next meeting.

Part 8 Records, privacy and information management

Records created under this document form part of the care record and are managed in accordance with the provider's Records Management Policy and the Privacy Act 1988 (Cth). Entries must be contemporaneous,

factual, legible and attributed to an identified author, and must not be altered after the fact except by a dated and attributed addendum.

8.1 Retention

Record	Minimum retention	Custodian
Medication charts	7 years after the last entry	Health Information Manager
Schedule 8 register	As required by State legislation, minimum 2 years	Director of Nursing
Medication incident records	7 years	Quality Coordinator
Refrigerator temperature logs	3 years	Nurse in charge
Medication review reports and outcomes	Life of the care record	Health Information Manager

8.2 Access and disclosure

An individual, or their supporter where authorised, may access records held about them on written request. Requests are actioned within 30 days. Personal information is disclosed outside the provider only with consent, or where disclosure is required or authorised by law, and every such disclosure is recorded in the disclosure register.

8.3 Information security

Access to the clinical information system is role-based and individually credentialled. Shared logins are prohibited. Access is revoked on the day a worker's engagement ends, and access logs are reviewed quarterly for anomalous patterns. A suspected data breach is escalated immediately under the Notifiable Data Breaches scheme.

Part 9 Monitoring, measurement and continuous improvement

Performance under this document is measured against the indicators below. The Director of Nursing reports results to the Medication Advisory Committee monthly and to the Board quarterly. An indicator that falls outside its target for two consecutive months triggers a documented improvement action with a named owner and a due date.

Indicator	Target	Source	Frequency
Residents prescribed an antipsychotic	below 10%	Quality indicator extract	Quarterly
Psychotropic orders with a documented indication	100%	Chart audit	Monthly
Medication incidents causing harm per 1,000 occupied bed days	0.2 or fewer	Incident system	Monthly
Schedule 8 counts completed at every shift change	100%	Register audit	Monthly
Medication reviews completed within 12 weeks of admission	at least 95%	Pharmacy report	Monthly
Refrigerator temperature excursions	Zero unresolved	Temperature logs	Weekly

9.1 Internal audit

The Director of Nursing conducts a documented internal audit of compliance with this document at least twice each year, using the tool at Appendix C. Findings are rated as conformity, opportunity for improvement, minor non-conformity or major non-conformity. A major non-conformity must be reported to the Medication Advisory Committee within five business days of identification.

9.2 Continuous improvement register

Improvement actions arising from audits, incidents, complaints, feedback and consumer advisory input are recorded in the continuous improvement register with an owner, a due date and an evidence field. The register is reviewed at each committee meeting and overdue items are escalated to the Board.

9.3 Consumer and workforce input

The provider seeks the views of individuals receiving care, their supporters and the workforce on the operation of this document at least annually, through the consumer experience survey, resident and relative meetings and workforce feedback channels. Results and the provider's response are published in the quarterly quality report.

Part 10 Risk management

Risks to the delivery of the requirements in this document are assessed using the enterprise risk matrix, which rates a risk on the likelihood of occurrence against the consequence for the individual receiving care. Risks rated high or extreme are recorded on the strategic risk register and reviewed by the Board each quarter.

Risk	Rating	Existing controls	Treatment
Psychotropic medicine used as a first response to changed behaviour	High	Behaviour support plan required within 5 days	Quarterly psychotropic audit; self-assessment by 31 March 2027
Crushing a modified-release preparation	High	Pharmacist written authority required	Crushing authority audit at each medication round review
Schedule 8 diversion by a worker	High	Two-person count each shift; safe within locked room	Unannounced counts introduced from 1 December 2026
Insulin dose error	High	Mandatory second-person check; annual calculation test	Independent double-check embedded in the electronic chart
Refrigerated medicine administered after a temperature excursion	Medium	Quarantine pending pharmacist advice	Automated temperature alarm to on-call phone by 30 April 2027
Verbal order not signed within 24 hours	Medium	Escalation to Director of Nursing	Weekly unsigned-order report from 1 February 2027

10.1 Dignity of risk

The provider supports each individual to make their own decisions, including decisions that carry risk. Where an individual chooses a course of action a clinician considers unsafe, the discussion, the information given, the individual's decision and any agreed mitigations are documented in the care plan and reviewed at each care plan review. Supporting a person's choice is not a failure of care.

Part 11 Review of this document

This document is reviewed at least every two years, and sooner where a legislative change, a major non-conformity, a serious incident, a coronial finding or a substantive change to practice makes review necessary. The next scheduled review falls due on 1 November 2028.

The review is coordinated by the Director of Nursing and must involve at least one registered nurse, one worker who applies the document in daily practice, and one consumer representative drawn from the Consumer Advisory Body. The reviewed document is approved by the Medication Advisory Committee and, where the change is material, by the Board.

11.1 Related documents

- KC-CLIN-009 Restrictive Practices and Behaviour Support Policy
- KC-CLIN-012 Clinical Deterioration and Escalation Procedure
- KC-CLIN-018 Infection Prevention and Control Policy
- KC-CORP-004 Incident Management Policy
- KC-CLIN-022 Advance Care Planning and End of Life Care Policy

11.2 Legislative and standards context

- Aged Care Act 2024 (Cth) and the Aged Care Rules made under it
- Strengthened Aged Care Quality Standards, effective 1 November 2025
- Privacy Act 1988 (Cth) and the Australian Privacy Principles
- Work Health and Safety Act 2011 (Cth) and corresponding state legislation
- Poisons and Therapeutic Goods legislation as applying in the relevant State or Territory

Part 12 Communication, engagement and supported decision-making

Every person receiving care from Kanangra Court Aged Care has the right to be told, in a way they understand, what is happening in their care and why. Information provided under this document must be offered in the person's preferred language and format, and must be repeated or restated where comprehension is uncertain. Reliance on a nod or a signature is not evidence of understanding.

12.1 Interpreters and communication support

An accredited interpreter must be engaged where a person's preferred language is not English and the discussion concerns consent, risk, a change to care, an incident affecting them or a complaint. Family members must not be used as interpreters for these discussions except in an emergency, and where that occurs the reason is recorded and the discussion repeated with an accredited interpreter within two business days.

12.2 Supported decision-making

The provider presumes that every person has decision-making capacity. Where capacity is in question, the assessment must be decision-specific, time-specific and documented, and must not be inferred from a diagnosis, age or communication difficulty. A person's supporter assists the person to make and communicate their own decision; a substitute decision-maker acts only where the person cannot make the decision even with support.

12.3 Engagement with supporters and representatives

The provider maintains a current record of each person's nominated supporters and representatives, including the scope of any authority held and the instrument granting it. The record is confirmed with the person at each care plan review and whenever the person's circumstances change.

12.4 Culturally safe practice

Care is delivered in a way that is safe for the person's culture, spirituality, gender, sexuality and life experience. For Aboriginal and Torres Strait Islander people, the provider offers connection to community-controlled health services and to family and community, and records the person's wishes about who is involved in decisions about their care.

Part 13 Emergency, contingency and business continuity

The requirements of this document continue to apply during an emergency. Where an emergency makes ordinary compliance impossible, the provider must record what was done instead, why, and for how long,

and must return to ordinary practice as soon as the emergency permits.

13.1 Planned system outages

Where the clinical information system is unavailable, workers record required information on the approved paper forms and enter it into the system within 24 hours of the system being restored. The paper record is retained and scanned into the care record; it is not destroyed after entry.

13.2 Surge, outbreak and staffing shortfall

During an infectious disease outbreak or a staffing shortfall, the Director of Nursing may authorise temporary variations to non-statutory requirements of this document. Statutory obligations, including notification timeframes, cannot be varied. Every temporary variation is recorded in the variations log with the authorising officer, the reason, the start date and the review date, and is reported to the next committee meeting.

13.3 Evacuation and relocation

Where a person is relocated in an emergency, the receiving service must be given the person's current care plan, medication chart, advance care directive and supporter contact details before or with the person. A relocation is not complete until a handover has been given to and acknowledged by a named clinician at the receiving service.

Part 14 Subcontracted and external service providers

The provider remains fully accountable for care delivered on its behalf by a subcontractor, agency or external practitioner. Engaging another organisation transfers work; it does not transfer accountability.

14.1 Contractual requirements

- Every service agreement must require compliance with this document by name.
- Every agreement must require the subcontractor to report incidents to the provider immediately, and in any event within four hours of becoming aware.
- Every agreement must permit the provider to audit the subcontractor's compliance on reasonable notice.
- Every agreement must require workers supplied to hold current screening and the qualifications the role requires.
- Every agreement must be reviewed before renewal, and no agreement may be renewed where a material non-conformity remains unresolved.

14.2 Monitoring subcontractor performance

The provider reviews the performance of each subcontractor at least annually against the indicators at Part 9, and more frequently where a concern has been identified. The review considers incidents, complaints, workforce screening currency and audit findings. The next scheduled subcontractor review cycle concludes on 31 March 2027.

14.3 Agency and locum workers

An agency worker may not commence a shift until the provider has sighted evidence of current registration where the role requires it, a current worker screening clearance, and completion of the provider's

orientation. The orientation attestation is retained for the duration of the engagement and for seven years afterwards.

Part 15 Mapping to the Strengthened Aged Care Quality Standards

This Part records how the requirements of this document give effect to the Strengthened Aged Care Quality Standards. It is used as the starting point for self-assessment and for evidence gathering before a quality audit.

Standard	Outcome	How this document gives effect to it
Standard 3	Care and services	Parts 2 and 4 require a valid order and a five-point check before every dose.
Standard 5	Clinical care	Part 5 requires review, deprescribing and psychotropic minimisation.
Standard 5	Medication safety	Part 3 governs storage, Schedule 8 control and disposal.
Standard 5	Restrictive practices	5.2 treats psychotropic use for behaviour as a restrictive practice.
Standard 2	The organisation	Part 9 measures medication safety indicators and reports them to the Board.
Standard 1	The individual	1.3 protects the right to refuse and restricts covert administration.

15.1 Evidence for audit

Evidence supporting each mapped outcome is maintained in the evidence library and indexed against the outcome number. Evidence must be current, dated and attributable, and must demonstrate practice rather than intention: a policy document alone is not evidence that the practice occurs.

15.2 Self-assessment

The Director of Nursing completes a self-assessment against the mapped outcomes at least annually and before any planned quality audit. The self-assessment is endorsed by the Medication Advisory Committee and reported to the Board. The next self-assessment is due by 30 April 2027.

Part 16 Assurance, external review and regulatory engagement

Assurance over this document operates on three lines: the workers and managers who apply it day to day, the Medication Advisory Committee which monitors and challenges performance, and independent review commissioned by the Board. Each line is documented and none substitutes for another.

16.1 Independent review

The Board commissions an independent review of compliance with this document at least every three years, or sooner following a major non-conformity or a serious incident. The reviewer must not have been involved in writing or operating the document. The report goes to the Board unaltered, together with

management's response. The next independent review must be completed by 30 September 2027.

16.2 Engagement with the Commission

The provider cooperates fully and promptly with the Aged Care Quality and Safety Commission. Requests for information are acknowledged within one business day and answered within the period specified, or an extension is sought in writing before the period expires. A worker approached directly by the Commission may speak with it freely; the provider must not require a worker to obtain permission first.

16.3 Responding to findings

Where a regulator or reviewer identifies a deficiency, the provider prepares a written response within 14 days setting out what will change, who is accountable, when it will be done and how effectiveness will be measured. Progress is reported to the Board monthly until every action is closed and verified.

16.4 Sanctions and notifiable changes

Where a sanction, a notice to agree, or a notice of non-compliance is issued, the Director of Nursing must notify the Board Chair on the day it is received, and the provider must tell affected individuals and their supporters what it means for their care within five business days.

Appendix A — Definitions

The following definitions apply throughout this document.

Term	Definition
PRN medicine	A medicine charted to be given as required rather than at a fixed time.
Psychotropic medicine	A medicine that affects mental processes, including antipsychotics, antidepressants, sedatives and anxiolytics.
Chemical restraint	A medicine used to influence a person's behaviour, other than for the treatment of a diagnosed condition.
Deprescribing	The planned and supervised process of dose reduction or cessation of a medicine that may be causing harm or no longer providing benefit.
Nurse-initiated medicine	A medicine on the approved list that a registered nurse may commence for up to 24 hours without a prescriber's order.
Temperature excursion	A recorded storage temperature outside the range the manufacturer specifies.

Appendix B — Competency assessment criteria

A worker is assessed as competent when they demonstrate each criterion below by direct observation in the workplace. The assessor records the date, the criterion, the outcome and any remediation required.

Criterion	Evidence sought	Outcome
Completes the five administration checks	Observed across five separate administrations	Competent / Not yet
Performs a Schedule 8 count and register entry	Observed count with correct documentation	Competent / Not yet
Identifies a medicine that must not be crushed	Correctly identifies modified-release preparations	Competent / Not yet
Responds correctly to a refused dose	Records refusal, explores reason, escalates appropriately	Competent / Not yet
Calculates a variable insulin dose	100% accuracy on the calculation test	Competent / Not yet

Appendix C — Internal audit tool

This tool is completed at each internal audit. Each question is answered yes, no or not applicable, with a comment recording the evidence examined. A 'no' answer requires a corrective action with an owner and a due date.

#	Audit question	Evidence examined
1	Does every charted medicine record an indication?	Medication charts
2	Were Schedule 8 counts completed and signed at every shift change in the sample period?	Schedule 8 register
3	Does every psychotropic order have consent and a behaviour support plan within 5 days?	Charts, behaviour support plans
4	Were verbal orders signed by the prescriber within 24 hours?	Medication charts
5	Were refrigerator temperatures within range, and were excursions actioned?	Temperature logs, incident records
6	Do crushing practices match a current pharmacist authority?	Charts, pharmacist authorities
7	Were medication reviews completed within 12 weeks of admission?	Pharmacy reports
8	Were antimicrobials reviewed within 72 hours with a documented stop date?	Charts, clinical notes

Appendix I — Abbreviations

The following abbreviations are used in this document and in the records it requires. Abbreviations must not be used in clinical entries where the meaning could be ambiguous.

Abbreviation	Meaning
ACQSC	Aged Care Quality and Safety Commission
ATAGI	Australian Technical Advisory Group on Immunisation
CGC	Clinical Governance Committee
CEO	Chief Executive Officer
EN	Enrolled nurse
IDDSI	International Dysphagia Diet Standardisation Initiative
MDT	Multidisciplinary team
PPE	Personal protective equipment
PRN	Pro re nata, meaning as required
RMMR	Residential Medication Management Review
RN	Registered nurse
SDM	Substitute decision-maker
SIRS	Serious Incident Response Scheme
WHS	Work health and safety

Appendix J — Worker acknowledgement

Every worker to whom this document applies must read it and acknowledge that they have done so. Acknowledgement records that the worker has read the document; it does not replace the competency assessment at Appendix B.

Acknowledgements are recorded in the learning management system. A worker who has not acknowledged the current version within 30 days of its publication is reported to their line manager, and a worker who has not acknowledged it within 60 days may not be rostered to a function this document governs until they do.

Acknowledgement statement

I confirm that I have read and understood this document, that I have had the opportunity to ask questions about it, and that I understand my obligations under it. I understand that where I am unsure how this document applies to a situation, I am expected to ask before acting rather than to assume.

Name	Role	Signature	Date

Completed acknowledgement sheets are returned to the document owner and retained for the life of the version plus seven years.

Appendix H — Implementation plan for the current version

The following milestones apply to the implementation of this version. Milestones are tracked by the document owner and reported to the committee monthly until all are closed.

Milestone	Owner	Due
Publish version 9.1 and withdraw earlier versions from all houses	Document owner	1 Nov 2026
Brief prescribers and the consultant pharmacist on the deprescribing pathway	Director of Nursing	20 Nov 2026
Complete competency reassessment for all workers who administer medicines	Clinical Educator	28 Feb 2027
Embed the independent double-check for insulin in the electronic chart	Digital Systems Manager	31 Mar 2027
First internal audit against the revised document	Quality Coordinator	30 Apr 2027
Report implementation completion to the Board	Director of Nursing	30 Jun 2027

Appendix E — Escalation matrix

This matrix sets out who must be told, and how quickly, when a matter arises under this document. Where two rows could apply, the shorter timeframe governs.

Trigger	Escalate to	Within
Suspected Schedule 8 diversion or theft	Director of Nursing, then police	Immediately
Medication incident causing harm	Prescriber and Director of Nursing	Immediately
Verbal order unsigned after 24 hours	Director of Nursing	Within 1 business day
Refrigerator temperature excursion	Consultant pharmacist	Same day
Psychotropic administered without a behaviour support plan	Clinical Nurse Consultant	Within 24 hours
Repeated refusal of a clinically significant medicine	Prescriber	Within 24 hours

Appendix F — Forms and templates

The following forms support this document. Forms are version-controlled and must be obtained from the intranet rather than copied from a local drive.

Form	Purpose	Retained by
KC-F-007a Medication incident report	Records the facts and clinical response to a medication incident	Quality Coordinator
KC-F-007b Schedule 8 register	Records receipt, administration, count and destruction	Director of Nursing
KC-F-007c Covert administration authority	Records consent, prescriber and pharmacist approval and review date	Clinical Nurse Consultant
KC-F-007d Self-administration risk assessment	Assesses a person's capacity to manage their own medicines	Registered nurse
KC-F-007e Crushing authority	Records pharmacist approval to alter a dose form	Consultant pharmacist

Appendix G — Frequently asked questions

These questions arose during consultation on the current version and are reproduced with the answers given.

Question	Answer
Can a care worker give a PRN paracetamol?	Only where the care worker is assessed as competent, the dose is pre-packed and unvarying, and the chart specifies the circumstances. PRN psychotropics may never be given by a care worker.
The prescriber is unavailable and the resident is in pain. What can I do?	A registered nurse may initiate a medicine from the nurse-initiated list at Appendix F for up to 24 hours, documenting the indication and notifying the prescriber at the next contact.
A resident spits out a tablet. Is that a medication incident?	Record it as a refusal and reassess. It becomes an incident where the dose was clinically significant and the omission was not escalated.
May I crush a tablet to help someone swallow it?	Only with a current written pharmacist authority recorded on the chart. Modified-release and enteric-coated preparations must never be crushed.
Who counts Schedule 8 medicines if only one registered nurse is on duty?	The count is performed by the registered nurse together with a second authorised worker. A single-person count is not permitted in any circumstance.

Appendix D — Improvement actions arising from the current review

The following actions were agreed at the most recent review of this document and are tracked in the continuous improvement register until closed.

Ref	Action	Responsible	Due
M1	Introduce unannounced Schedule 8 counts	Director of Nursing	1 Dec 2026
M2	Review every covert administration authority in force	Clinical Nurse Consultant	31 Jan 2027
M3	Weekly report of verbal orders unsigned beyond 24 hours	Quality Coordinator	1 Feb 2027
M4	Complete the provider-wide psychotropic self-assessment	Director of Nursing	31 Mar 2027
M5	Install automated refrigerator alarms linked to the on-call phone	Facilities Manager	30 Apr 2027
M6	Publish the annual antimicrobial stewardship report	Medication Advisory Committee	31 Jul 2027